

The background of the entire cover is a photograph of a man in a rural Ethiopian landscape. The man, who has a grey beard and is wearing a patterned grey shirt and a dark red shawl, stands in a lush green field of tall grass or crops. He is holding a wooden staff. In the background, there are rolling green hills, some trees, and a few small buildings under a cloudy sky. Another person is visible working in the field in the distance.

EHF Ethiopia
Humanitarian
Fund

ETHIOPIA HUMANITARIAN FUND

2025

ANNUAL REPORT

Credits

This document was produced by the United Nations Office for the Coordination of Humanitarian Affairs (OCHA) Ethiopia. OCHA Ethiopia wishes to acknowledge the contributions of its committed staff at headquarters and in the field in preparing this document.

The latest version of this document is available on the EHF website at

<https://www.unocha.org/ethiopia-humanitarian-fund>

Full project details, financial updates, real-time allocation data and indicator achievements against targets are available at [CBPF DataHub](#).

About EHF

Front Cover

A farmer stands in his wheat field, supported by ZOA and ASDEPO NGOs with EHF funding.

22 November 2025,
Woldiya town, North Wello Zone,
Amhara Region.

Photo: UNOCHA/Petro Riabukhin

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LETTER FROM THE HUMANITARIAN COORDINATOR

In 2025, Ethiopia continued to face a complex humanitarian situation shaped by the combined effects of conflict, climate shocks, displacement and disease outbreaks. Millions of people remained in need of critical assistance, particularly in areas affected by insecurity, recurrent drought and limited access to essential services.

In this context, the Ethiopia Humanitarian Fund (EHF) continued to play a central role in supporting life-saving humanitarian assistance across the country. Through allocating US\$44.2 million, the Fund enabled humanitarian partners to reach approximately 3.2 million people, including 1.8 million women and girls and nearly 200,000 people living with disabilities. This impact was made possible through the continued support of 12 donors, whose valuable contributions remain vital to sustaining assistance for the most vulnerable.

Humanitarian operations in Ethiopia in 2025 unfolded against a backdrop of increasing global funding constraints. At a time when humanitarian needs remain high and increasingly complex, and available resources continue to decline, the EHF once again demonstrated its value as a flexible and responsive funding mechanism, capable of directing resources where they are needed most. With a strong network of pre-assessed partners, the Fund enabled rapid allocation and implementation, while maintaining robust standards for risk management, compliance and accountability.

All EHF allocations in 2025 were aligned with priorities set by the Humanitarian Country Team and Advisory Board and informed by regular Priority Humanitarian Response and Critical Funding Gaps analyses led by the Inter-Cluster Coordination Group in close coordination with field-level structures. This collective approach ensured that limited resources were directed to the most urgent, life-saving needs, based on inter-sectoral shared analysis and evidence.

Throughout the year, EHF supported 67 projects implemented by 47 non-governmental organizations, delivering assistance to vulnerable communities across multiple regions. The Fund

enabled an integrated response to address food insecurity-, malnutrition-, health-, water-, sanitation- and hygiene-, education-, and protection-related needs, including gender-based violence, as well as the needs of internally displaced people. Importantly, the EHF funding reached people in hard-to-reach and conflict-affected areas, where humanitarian access remains constrained, and assistance would otherwise be limited.

The Fund also remained a key instrument for advancing localization in Ethiopia. In 2025, more than 80 per cent of funding was allocated to Ethiopian non-governmental organizations. Nearly 34 per cent of the funding was channelled to women-led organizations. These efforts contribute to strengthening local leadership and ensuring that humanitarian response is more inclusive, accountable and grounded in the realities of affected communities.

I would like to express my sincere appreciation to all donors to the Ethiopia Humanitarian Fund for their continued support and trust. I also thank the members of the Advisory Board – United Nations agencies, non-governmental organizations and donor representatives – for their guidance and engagement. My appreciation also goes to cluster coordinators, strategic review committees and the OCHA team for their dedicated stewardship of this critical funding mechanism.

At a time of persistent needs and constrained resources, continued solidarity and collective action are more essential than ever. Together, we must sustain our efforts to ensure that life-saving assistance reaches those who need it most, wherever they are.



Sincerely,

Ozonnia Ojielo
Humanitarian Coordinator for Ethiopia

A photograph of a man in the foreground carrying a large white sack of rice on his head. The sack has a red logo and text including "AID FUTURE", "RICE", and "COFCO CORPORATION". He is wearing a blue headscarf and a grey jacket. In the background, other people are visible, including a man in a yellow cap and a woman in a patterned headscarf. The scene is outdoors under a blue sky with clouds.

“ At a time when humanitarian needs remain high and increasingly complex, and available resources continue to decline, the EHF once again demonstrated its value as a flexible and responsive funding mechanism, capable of directing resources where they are needed most ”

Ozonnia Ojielo
Humanitarian Coordinator for Ethiopia

Bakelo camp residents are helping to offload food assistance. Bakelo camp hosts several thousand displaced people.

May 2025, Debre Berhan town, Amhara Region.

Photo: UNOCHA/Charlotte Cans

2025 IN NUMBERS



\$32.8M
CONTRIBUTIONS

Contributions in US\$



3.2M
PEOPLE
ASSISTED



195k
PEOPLE WITH
DISABILITIES ASSISTED



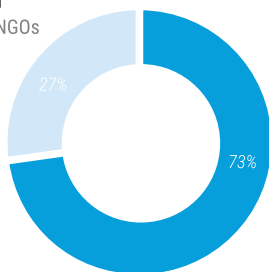
1.7M
WOMEN AND GIRLS ASSISTED
55% OF TOTAL PEOPLE ASSISTED



47
PARTNERS

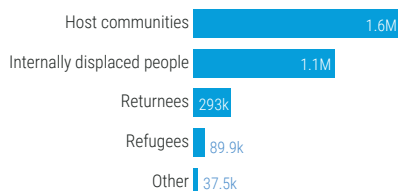
67
PROJECTS

\$11.9M
International NGOs
16 Partners
19 Projects



Allocations in US\$

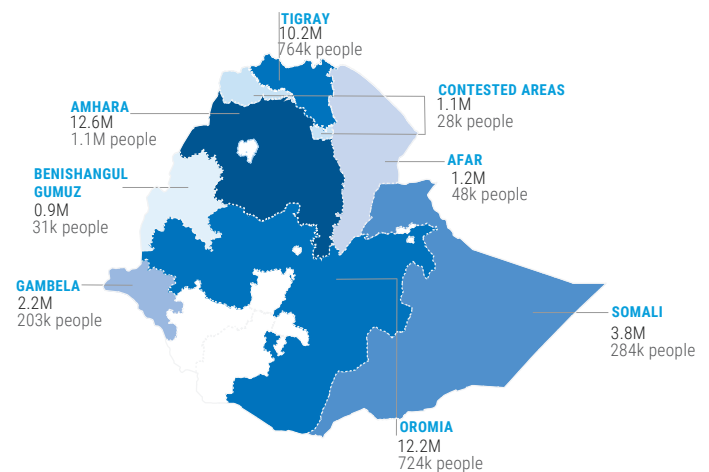
PEOPLE ASSISTED BY TYPE



\$44.2M
ALLOCATIONS

3.2M
PEOPLE ASSISTED

Allocations in US\$



The boundaries, names and designations used do not imply official endorsement or acceptance by the United Nations. References to 'contested areas' are understood within the context of the Pretoria Agreement [Article 10, 2022].

The Annual Report uses the number of people targeted as a proxy for the number of people reached and henceforth the term people assisted will be used. This approach allows for more timely global reporting as the final data on people reached only becomes available over a year after the allocation of CBPF funds. The reported outcomes will be available on the <https://cbpf.data.unocha.org/> the CBPFs will continuously monitor if targets are reached.

Figures for people assisted may include double counting as individuals often receive aid from multiple cluster/sectors. The maximum methodology was applied by the Ethiopia Humanitarian Fund to estimate the number of people assisted in 2025. The total number of people assisted is thus the sum of the maximum number of targeted beneficiaries by gender and age by project and location at admin level three/district.

	Allocations		Total	People Assisted
Protection	5.0	5.1	10.1	0.5
WASH	2.1	5.3	7.4	1.1
Health	1.9	4.4	6.3	1.1
Nutrition	2	4.2	6.2	0.6
ESNFI	4.6		4.6	0.1
Education	4		4	0.08
Food	2	1	3	0.2
MPC	1.7		1.7	0.02
CCCM	0.9		0.9	0.3

Standard allocations Reserve allocations

Allocations in US\$ million, people assisted in millions



379k
people with access to
safe drinking water
through emergency
water supply



94k
displacement-affected
people received
emergency shelter
and non-food items



19k
children received
teaching and learning
materials and assistive
devices



29k
people received
services, including
mental health and
psychosocial
support

Donor contributions

In 2025, the Ethiopia Humanitarian Fund (EHF) received US\$32.8 million from 12 donors, a decrease of \$29 million, or approximately 47 per cent, compared with \$61.8 million from 16 donors in 2024. This sharp decline reflects the wider contraction in humanitarian financing globally and the increasingly constrained environment in which country and regional funds operate. Despite reduced funding, donor support enabled the EHF to continue directing resources to address the most urgent humanitarian needs in Ethiopia, in line with the Humanitarian Coordinator's priorities.

However, a significant funding reduction in 2025, coupled with an already 18 per cent decline in 2024 funding compared to 2023, led to less flexibility in allocation decisions, forcing tighter prioritization and limiting the Fund's ability to respond at scale. It also constrained opportunities to expand support to national partners and advance localization, highlighting the direct impact of declining contributions on both operational reach and strategic objectives.

The top five donors to the EHF in 2025 were the Netherlands (\$10.3 million), Denmark (\$4.7 million), Finland (\$4.6 million), Canada (\$3.6 million) and Ireland (\$3.6 million). Additional contributions were received from Sweden (\$2.5 million), Norway (\$1.3 million), New Zealand (\$900,000), Luxembourg (\$700,000), Switzerland (\$500,000), the Philippines (\$20,000) and UN and other agencies (\$5,000). The contribution from the Philippines signals continued scope for broadening the EHF donor base beyond its traditional supporters.

The EHF will continue to intensify donor outreach in 2026, with a focus on strengthening strategic engagement, improving the visibility of results and demonstrating the Fund's added value as a flexible, locally anchored and high-impact financing mechanism. Priorities will include stronger evidence-based communication on results and impact, continued facilitation of

Key Figures

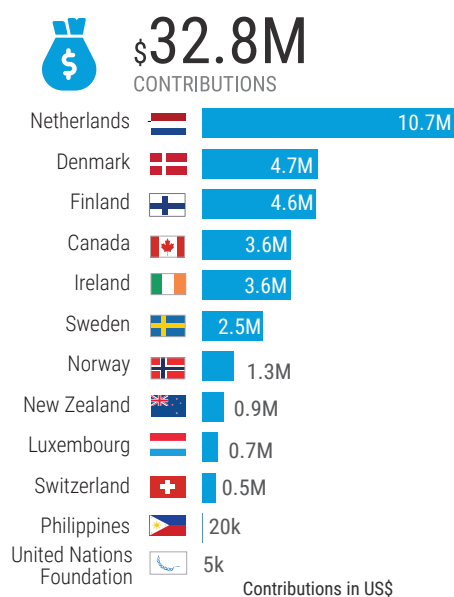


*Recommended by the Advisory Board and endorsed by the Humanitarian Coordinator

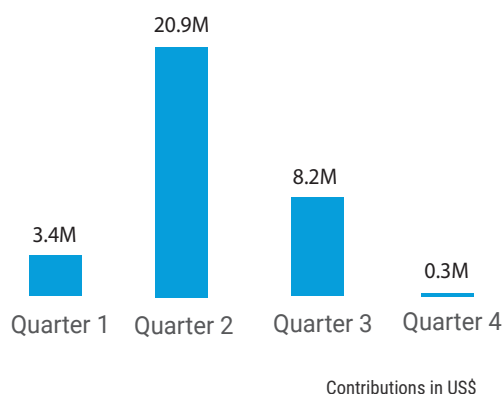
donor field missions, and closer engagement on complementarity with CERF and other funding streams. Maintaining and expanding a diverse donor base will also be important as the Fund navigates continued uncertainty in the humanitarian financing landscape.

Since its establishment in 2006, the EHF has been one of the largest country-based pooled funds globally, ranking ninth in 2025. By the end of 2025, and since its inception, it had received \$1 billion, with Germany, the Netherlands and

DONOR CONTRIBUTIONS



CONTRIBUTIONS BY QUARTER



donor base had grown from 8 donors in 2017 to 12 in 2025, with a total number of countries having contributed to the Fund in different years reaching 27; and 2025 confirmed the need for a broader, more diversified donor base to preserve the Fund's scale and flexibility in the years ahead.

DONORS WITH MULTI-YEAR CONTRIBUTIONS

	Canada (2025 - 2026)	\$3.6M
	Ireland (2023 - 2025)	\$3.4M
	Switzerland (2023 - 2026)	\$0.5

Contributions in US\$

the United Kingdom consistently among its largest contributors, in line with global trends, where these donors collectively account for a substantial proportion of pooled funding. The

“Flexible funding through the Ethiopia Humanitarian Fund allows partners to respond rapidly to the most urgent needs while ensuring strong coordination and accountability”

Ms. Sinikka Antila, Ambassador of Finland to Ethiopia, Djibouti and South Sudan and Permanent Representative to African Union, IGAD and UNECA

Women getting clean water from a solar-powered borehole system. The system was installed by GOAL NGO with EHF funding and includes two water points, a reservoir, a cattle trough, and a clothes-washing spot for 17,000 people.

March 2025,
Borena Zone,
Oromia Region.

Photo: UNOCHA/Manuel Morini



Humanitarian Context and Allocations

Humanitarian context

Throughout 2025, the humanitarian situation in Ethiopia remained complex, shaped by the combined effects of climate variability, localized insecurity, disease outbreaks, displacement, and broader economic pressures. These interrelated factors affected vulnerable populations, stretched coping capacities, and impacted access to essential services and livelihoods, particularly in areas already facing structural challenges.

The situation evolved over the course of the year, influenced by seasonal patterns and localized security dynamics. Early 2025 was marked by below-average and delayed Deyr/Hageya rains in southern and south-eastern lowland areas, associated with La Niña conditions. This contributed to drought-like conditions in pastoral and agro-pastoral zones, particularly in Somali and Oromia regions, where communities were still recovering from previous climatic shocks. As a result, food insecurity and acute malnutrition remained areas of concern, with many households experiencing reduced livestock productivity, limited market access, and increased pressure on livelihoods.

Later in the year, localized flooding and landslides in other parts of the country further affected communities, damaging infrastructure, displacing populations, and interrupting service delivery. These climate-related events highlighted the need for context-specific humanitarian responses, including anticipatory and early action approaches.

Localized insecurity in some regions, including parts of Amhara and Oromia, as well as areas along regional borders, continued to impact communities by contributing to displacement, disrupting access to basic services, and affecting the operating environment. The fluid nature of the situation posed challenges for consistent service delivery, particularly in remote and hard-to-reach locations. Protection concerns remained significant in affected areas, including risks related to gender-based violence, child protection, and exposure to explosive hazards, especially among displaced populations.

Public health challenges also influenced the humanitarian context. Cholera outbreaks were reported in multiple regions, linked to displacement, water and sanitation conditions, and climatic factors. Malaria and measles cases fluctuated throughout the year, particularly in Oromia, Amhara, and southern regions, with access to prevention and treatment services varying across locations. In November 2025, Ethiopia confirmed its first Marburg virus disease outbreak, which the Government quickly contained, underscoring the importance of continued preparedness and a coordinated response to emerging health risks.

Population movements remained dynamic, driven by a combination of climatic pressures and localized insecurity. Internal displacement persisted across several regions, including Amhara, Oromia, Tigray, and Somali. At the same time, return movements remained gradual, influenced by security conditions, access to services, and local dynamics. Ethiopia also continued to host a significant number of refugees, placing additional demand on services in host communities.

Across contexts, humanitarian operations continued to face a range of operational challenges, including security considerations, administrative processes, and logistical constraints, which at times affected the timely delivery of assistance, particularly in remote and underserved areas. In this context, strengthening collaboration with national and local partners remains essential to ensure sustained and effective support for affected populations.

2025 ALLOCATIONS

First Reserve Allocation

Launched in April 2025, the First Reserve Allocation provided US\$5.2 million to address urgent humanitarian needs driven by cholera outbreaks, malnutrition and protection risks, at a critical time when funding disruptions threatened the continuity of essential services.

The Ethiopia Humanitarian Fund (EHF) fast-tracked the following humanitarian interventions through this allocation:

- Cholera response (\$1.7 million to support around 250,000 people)
- Emergency health and nutrition services (\$1.5 million to assist nearly 150,000 people)
- Gender-based violence-related services (\$2 million to support 50,000 vulnerable people).

The allocation aimed to provide integrated assistance to 420,000 people with overlapping needs across Amhara, Oromia, Tigray, Afar and Gambela, including improved access to sanitation facilities and safe drinking water for 120,000 people, improved health services through primary health-care consultations and family planning for 60,000 people, supplements for 40,000 children through outpatient therapeutic programmes and other nutrition support, basic supplies to address cholera, emergency mental health and psychological support, services for gender-based violence survivors and prevention activities, emergency cash and other assistance.

The allocation also reinforced the role of national partners in humanitarian action. Ethiopian NGOs (9 out of 12 partners) received 73 per cent of funding (\$3.8 million), enabling them to assist people in hard-to-access areas where operational presence and community trust were critical.

First Standard Allocation

The First Standard Allocation, launched in May 2025, provided US\$17 million to address high-severity and protracted humanitarian needs across Amhara, Oromia, Somali, Tigray and Gambela, where conflict, displacement, food insecurity and protection risks continued to affect large segments of the population.

The allocation prioritized three areas:

- Integrated response to malnutrition (\$8 million)
- Response to critical gaps in gender-based violence services (\$5 million)
- Access to emergency education for crisis-affected communities (\$4 million)

Projects under this allocation aimed to support nearly 950,000 vulnerable people, including malnourished children, pregnant and lactating women, gender-based violence survivors and out-of-school children. Assistance included therapeutic feeding and nutrition support, access to health and related services, psychosocial and protection support, and the provision of learning materials and safe learning environments, helping stabilize conditions for families facing prolonged crisis.

Representing approximately 38 per cent of EHF funding in 2025, this was the Fund's largest allocation and a lifeline in areas with persistent humanitarian needs. Twenty-three partners (out of them 18 national organizations) implemented projects, including to assist 94,000 people with sanitary and hygiene kits, 80,000 people with animal health interventions, more than 60,000 children and pregnant women with vitamins and micronutrients, at least 55,000 people with clean water, over 40,000 children with access to learning and recreational materials, 16,000 survivors of gender-based violence and vulnerable women with case management and more. Some \$750,000 in cash-based interventions was allocated for disbursement to 13,000 people across regions.

Second Reserve Allocation

Launched in April 2025, when the EHF provided US\$6 million to address critical, life-threatening humanitarian needs in the Tigray Region, amid deteriorating conditions linked to sustained displacement, funding shortfalls, collapsing service pipelines and heightened vulnerability ahead of the rainy season.

The funding was allocated to provide:

- Multi-sectoral assistance for internally displaced people in collective sites (\$2.5 million to support nearly 345,000 people)
- Integrated response to protection risks, including gender-based violence (\$1.5 million to assist 95,000 people)
- Nutrition response in hotspot areas (\$2 million to support 200,000 people).

These interventions enabled 640,000 vulnerable people across north-western, central and eastern zones of Tigray to receive integrated assistance, particularly in collective sites and underserved areas where access to services remained limited. Support included improvements in living conditions through shelter and site management, access to safe water and sanitation, treatment and prevention of malnutrition, primary health-care services, and protection interventions to reduce risks and improve safety and dignity for displaced people.

Under this allocation, Ethiopian NGOs received 75 per cent of the funding (\$4.5 million), enabling them to ensure the continuity of essential services during a period of heightened vulnerability.

Third Reserve Allocation

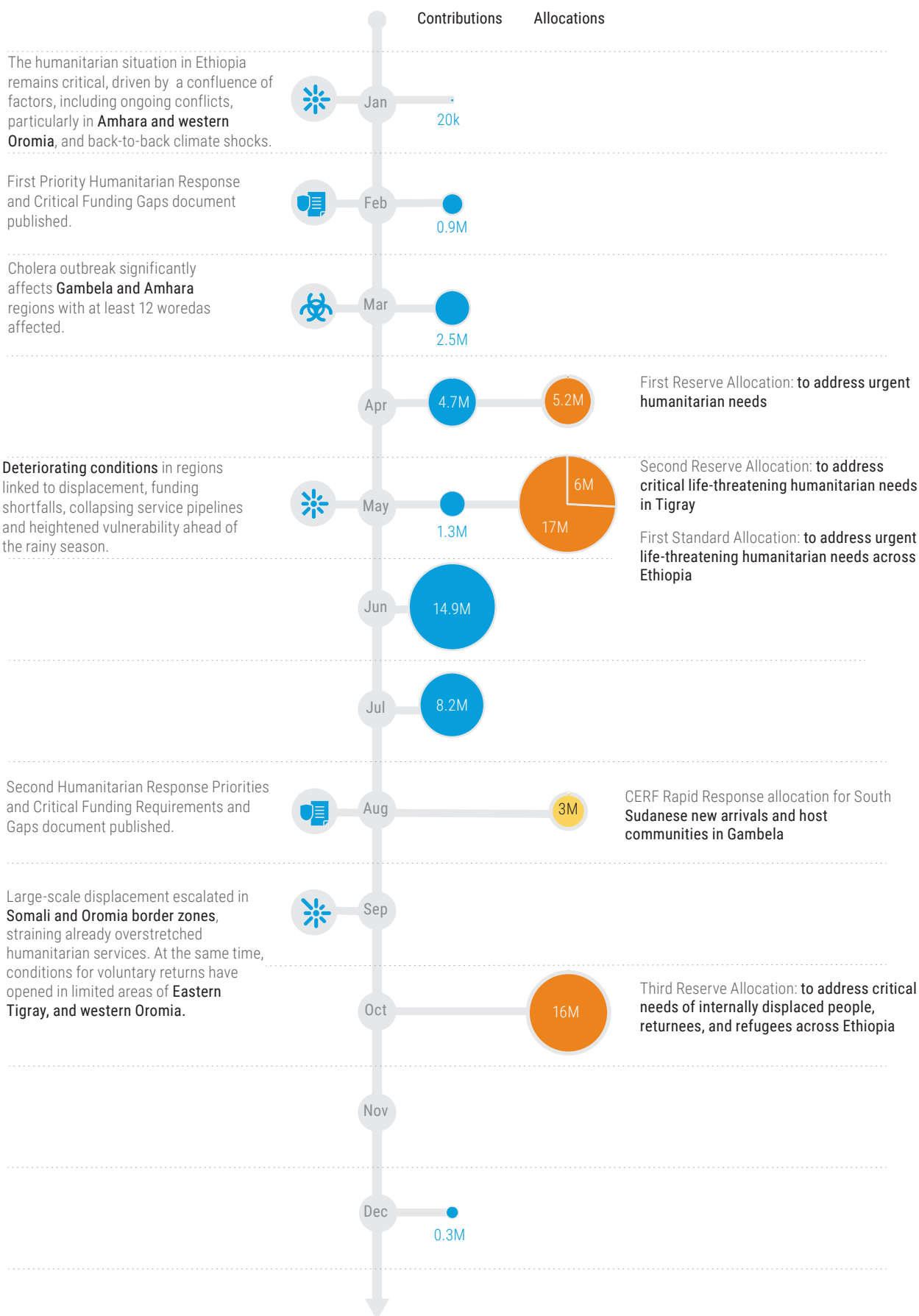
The Third Reserve Allocation, launched in October 2025 with US\$16 million, aimed to support 1.16 million people in the context of ongoing conflict, displacement and disease outbreaks. The allocation focused on:

- Multi-sectoral support in prioritized sites for internally displaced people, including collective centres in Amhara, Oromia and Tigray, and informal settlements in Somali and Oromia, with particular emphasis on border conflict-affected zones (\$10.5 million)
- Time-bound humanitarian assistance in areas of voluntary return of internally displaced people to Eastern Tigray and Western Oromia (\$4.5 million)
- Assistance to unregistered refugees from South Sudan living near host communities in Gambela, providing a six-month humanitarian package to support their transition into formal refugee camps under UNHCR's mandate (\$1.5 million).

Affected people across multiple regions received integrated assistance, particularly in displacement settings and underserved areas where needs had intensified, and access to services remained constrained. Support included improved access to basic services in collective sites and informal settlements, assistance for returning internally displaced people to meet immediate needs during transition, and sustained support for vulnerable refugees through coordinated multi-sectoral packages.

The EHF continued to strengthen the localization agenda. By enabling 22 partners to rapidly scale up assistance, including national organizations operating in hard-to-reach areas, the allocation helped maintain the continuity of life-saving services and respond to emerging needs in a highly dynamic operational environment.

2025



A man receives household water treatment chemicals distributed by Fayyaa Integrated Development Organization with the financial support of EHF to support safe water access for conflict-affected families.

December 2025,
Diga Woreda, East Wellaga
Zone, Oromia Region.

Photo: FIDO/Galena Tasfa



Implications for humanitarian response and the Ethiopia Humanitarian Fund (EHF) allocations

In the context of declining humanitarian funding, prioritization focused on life-saving interventions, high-severity areas and locations with limited humanitarian presence. This prioritization approach meant that assistance could only reach a portion of people in need, reinforcing the importance of targeted allocations and complementary funding streams. In 2025, the EHF aimed to support 3.2 million people across Ethiopia.

The EHF allocations were designed to address existing humanitarian needs, including acute malnutrition, disease outbreaks and protection risks; support multi-sectoral response to displacement; enable rapid response to emerging shocks; ensure complementarity with CERF and other funding streams; and prioritize local and national responders.

The sequencing of allocations aligned with the evolution of needs: earlier reserve allocations addressed outbreak impacts, followed by a standard allocation targeting high-severity and underserved areas, and a late-year reserve allocation responding to conflict, displacement and disease outbreaks.

Overview of the EHF allocations in 2025

In 2025, the EHF launched four allocations totalling US\$44.2 million, comprising three reserve allocations and one standard allocation. These allocations were sequenced to respond to emerging shocks and critical humanitarian needs. The allocations targeted approximately 3.2 million people, of whom around 1.62 million people (51 per cent) had been reached by the end of the year, while many projects were still ongoing moving into 2026.

The Fund prioritized multi-sectoral response integrating health, WASH, nutrition, emergency livelihoods, protection and emergency shelter interventions; displacement-affected people; conflict-affected and hard-to-reach areas; gender-transformative programming, prevention and response to gender-based violence; and localization, with emphasis on national and women-led organizations.

The EHF promoted area-based and community-informed approaches, which prioritized geographically targeted response, where needs were most acute, and enabled partners to design multi-sectoral interventions based on localized

Community members receive food assistance at Asbole-Boda evacuation site as part of the humanitarian response to evacuations due to a surge in seismic activity in nearby areas.

January 2025,
Awash Woreda,
Afar Region.

Photo: UNOCHA/Alex O'Mahony

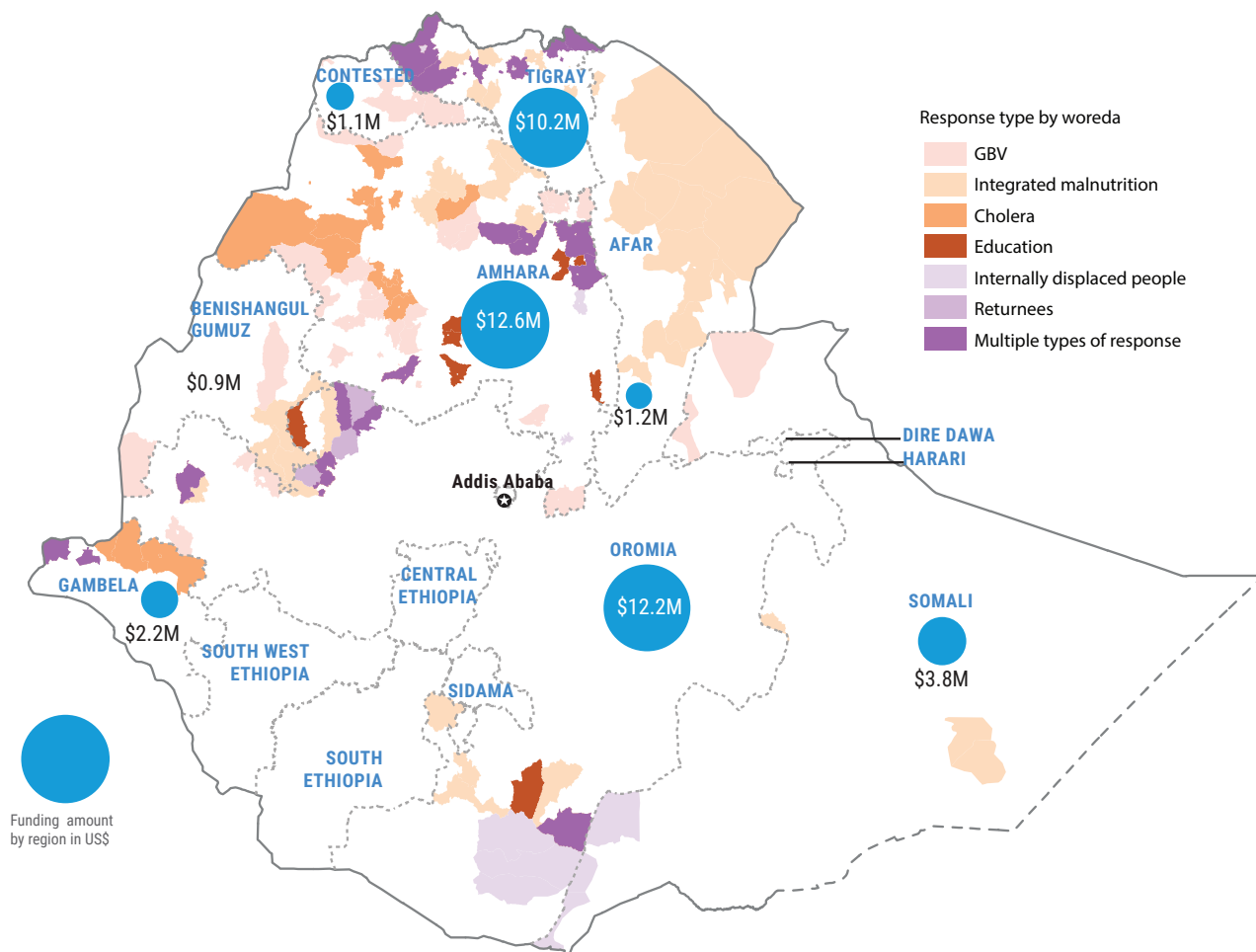


analyses of vulnerabilities and service gaps. Subnational coordination structures and clusters played a central role in identifying priorities, with review processes incorporating expertise and inputs from local actors to ensure that proposed interventions reflected operational realities.

This helped strengthen the relevance, coherence and timeliness of assistance, while bringing decision-making closer to affected communities. Around 77 per cent of funding (\$34 million) was channelled directly to implementing partners. In comparison, some 23 per cent (\$10.2 million) was delivered through sub-implementing arrangements, supporting both efficient delivery and broader operational reach.

Through this combination of reserve and standard allocations, the Fund supported life-saving interventions in high-severity and hard-to-reach areas, with a focus on displacement, protection risks, disease outbreaks and food insecurity. The EHF increasingly prioritized empowering affected people through integrated context-specific assistance, including cash-based support and community-informed approaches, to reduce dependency on humanitarian aid and strengthen resilience.

SUMMARY OF EHF ALLOCATIONS IN 2025



This region-by-region breakdown of the EHF allocations in 2025 reflects the concentration of humanitarian aid and highlights the Fund's focus on conflict-affected, displacement-heavy and climate-vulnerable regions, as well as areas with limited humanitarian presence.

Promoting Localization



The Ethiopia Humanitarian Fund (EHF) continued to advance localization in 2025 by prioritizing direct funding to national partners and strengthening locally led humanitarian action. In line with its Localization Strategy published in July 2025, the Fund remained committed to increasing funding for national and local responders, promoting equitable partnerships and supporting the institutional capacity of national actors to lead humanitarian response. In 2025, the EHF further consolidated its position as a principal source of humanitarian funding for national actors in Ethiopia, reinforcing its role in enabling locally led response.

Increased direct funding to local partners

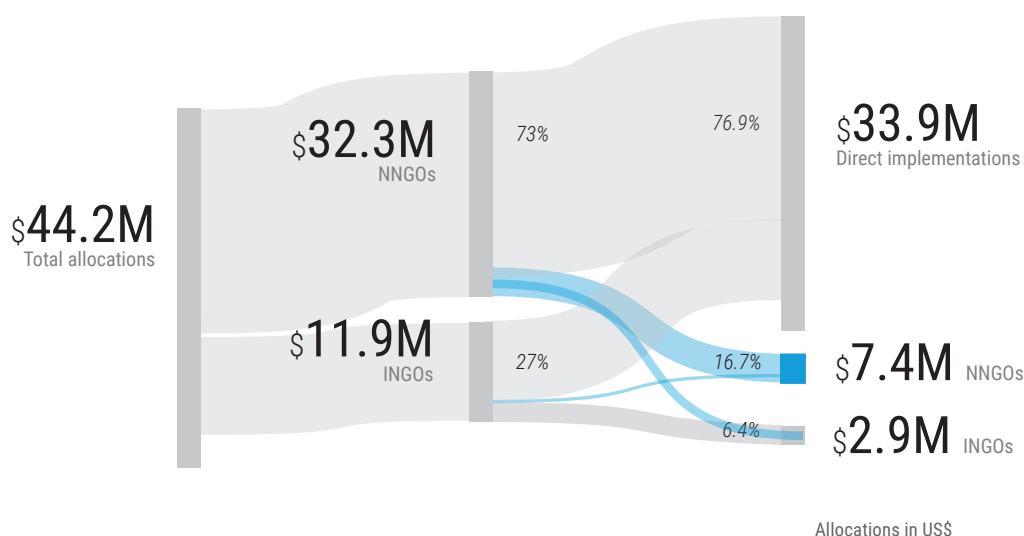
In 2025, approximately US\$32.3 million (73 per cent) of funding was channelled directly to national partners, with a further \$3.3 million (7.5 per cent) reaching sub-implementing partners

national NGOs. National partners played a central role in delivering assistance, particularly in hard-to-reach and access-constrained areas.

Across reserve allocations, localization remained a consistent feature. For example, in the First Reserve Allocation, national NGOs received 73 per cent of funding (\$3.8 million), while in the Second Reserve Allocation, they received 75 per cent (\$4.5 million), demonstrating the Fund's ability to translate localization commitments into operational practice and confirming the Fund's strong emphasis on channelling resources to national actors.

Progress over time further illustrates this shift. Overall funding to national NGOs increased from 12 per cent in 2021 to 81 per cent in 2025.

ALLOCATION FLOW BY PARTNER TYPE



Stabilization Center (SC) unit established by Fayyaa Integrated Development Organization as part of a multisectoral emergency response project funded by the EHF.

May 2025, Fitch town, North Shewa Zone, Oromia Region.

Photo: FIDO/Dawit Mulugeta



A national organization, Fayyaa Integrated Development Organization (FIDO), provided water-, sanitation- and hygiene-related assistance in Jido Misira, East Hararghe, an area where communities previously relied on unsafe water sources and that has been hard to reach for most humanitarians. Through the installation of a solar-powered water system (28 panels, 15,400W capacity), the project provided access to safe water for more than 10,000 families, reducing reliance on distant and unsafe sources. The intervention contributed to a reported 50 per cent reduction in waterborne diseases, reduced the time spent on water collection, particularly for women and children, and strengthened local capacity through training for system operators. This highlights the ability of national partners to deliver effective assistance in access-constrained environments.

Diversified partner base

The number of national partners receiving direct funding grew from 5 in 2021 to 32 in 2025, with 11 new NGOs obtaining funding for the first time. At the same time, funding to women-led organizations increased more than fivefold since 2021 – from five to almost 34 per cent in 2025 – reflecting efforts to broaden the participation of diverse local actors. As of 2025, the EHF has 18 eligible partners among women-led organization, 10 of which were onboarded in 2025. Five out of these newly onboarded partners received funding for the first time to implement seven projects in hard-to-reach areas, including cholera response and assistance to returnees in areas bordering Eritrea. Many of these organizations focus on protection, gender-based violence and inclusive programming, reinforcing the role of women-led actors in addressing priority humanitarian needs in displacement-affected and high-risk areas.

Partner diversification also improved, with the share of funding received by the ten largest

partners declining from 50 per cent in 2021 to 19 per cent in 2025, indicating a more distributed funding model. This trend is reflected in 2025 allocations, where national NGOs accounted for over 81 per cent of total funding. In total, 47 partners implemented 67 projects in 2025, reflecting an increasingly diversified and inclusive partner base. In 2025, the EHF prioritized onboarding partners working in hard-to-reach and underserved areas where local responders on the ground had better access and greater efficiency in reaching people in need.

To further strengthen the voice and representation of national actors, the governance structure of the Fund was reviewed in 2025, in accordance with the Localization strategy, further agreed on by the Advisory Board and endorsed by the Humanitarian Coordinator, to include five national NGO networks in advisory and decision-making mechanisms.



A women-led organization, the New Millennium Women Empowerment Organization (NMWEO), implemented a protection project in Central Tigray that shifted from one-time dignity kit distribution to a sustainable, women-led production model for reusable sanitary pads, combining protection outcomes with livelihoods support. Through this approach, more than 1,000 internally displaced women and girls accessed menstrual hygiene products, while over a dozen vulnerable women were trained and employed to produce reusable pads, establishing a locally owned cooperative model. The initiative improved access to essential items, reduced long-term costs and created income opportunities, demonstrating how women-led organizations can deliver integrated humanitarian response that strengthens dignity, economic empowerment and sustainability.

Quality response

Monitoring data provided evidence that increased localization had not come at the expense of quality. Among national NGO partners categorized as high or medium risk, 87 per cent received monitoring ratings of “good” or above, including in complex operational environments. This indicated that national partners were increasingly able to deliver humanitarian response at scale and in line with required standards, including in areas with limited access and heightened risks. This was consistent with overall monitoring results, which showed that the majority of projects across risk levels met the required standards for implementation and reporting.

Structural barriers and equitable partnerships

At the same time, structural constraints continued to affect the depth of localization. While more national partners were accessing funding, many newly onboarded organizations (more than half of 40 onboarded since 2021) had implemented only one or two projects, limiting their ability to scale and highlighting the challenge of moving beyond initial engagement towards sustained partnership.

Funding ceilings linked to partner risk categories remained a key constraint, even where partners demonstrated strong performance. In addition, the share of indirect funding through sub-granting arrangements decreased to 7.5 per cent, underscoring the Fund’s efforts to eliminate pass-through arrangements and provide funding as directly as possible to local responders. In 2025, the EHF continued to address risk-sharing challenges by expanding direct funding to national partners while maintaining strengthened oversight and compliance frameworks. The share of indirect funding through sub-granting arrangements decreased to 7.5 per cent, underscoring the Fund’s efforts to reduce intermediary layers and ensure that funding reaches frontline responders more directly.

Supported by the Advisory Board and led by the Humanitarian Coordinator, efforts were also made to promote more equitable partnership arrangements, including fairer sharing of programme support costs. Out of 67 projects allocated in 2025, 49 projects included sub-implementing partners, with a 7 per cent programme support cost share allocated proportionally to all sub-partners. While this represented progress towards more balanced partnerships, continued efforts were needed to ensure more equitable cost-sharing arrangements and strengthen the financial sustainability of national organizations. Duty of care considerations were also integrated into programme design and implementation, including through capacity-strengthening, risk mitigation measures and strengthened support to partners operating in high-risk and access-constrained environments.

Capacity-strengthening and innovation

A dedicated Partnerships Officer position was established to support the implementation of the Localization Strategy, including analyzing partner capacity assessments and developing structured capacity-strengthening plans. The Fund also explored mechanisms to support mentorship arrangements between experienced and newly onboarded partners, as well as the allocation of targeted resources within projects to address identified capacity gaps. OCHA invested in strengthening the capacity of national actors, training over 2,500 national humanitarian workers to enhance the technical and operational capacities of local partners and support more effective delivery of the humanitarian response.

The EHF funding also supported locally led and innovative approaches that combined humanitarian assistance with early recovery and resilience-building. For example, a women-led initiative implemented by a national organization

in Tigray shifted from the one-off distribution of dignity kits to the local production of reusable sanitary pads through a cooperative model. By combining protection outcomes with income generation and local ownership, the initiative demonstrated how locally led solutions can

address immediate needs while contributing to longer-term resilience and sustainability.

Local residents collect water at a communal water point rehabilitated by a national NGO Mothers and Children Multisectoral Development Organization with funding from the EHF to support more than 8,500 people with access to safe water.

July 2025,
May Tsebri town,
Contested Areas.

Photo: UNOCHA/ Manuel Morini



“ The support of the Ethiopia Humanitarian Fund demonstrates the transformative impact of investing in local leadership. Through this partnership, organizations like ours are not only delivering life-saving assistance but also helping shape humanitarian response that are grounded in the realities of the communities we serve. What makes this partnership particularly meaningful is the spirit in which it is delivered, marked by transparency, timely support, genuine collaboration, and a deep commitment to reaching communities who are in need of urgent care and support

Reverent Anbessu Tolla
Executive Director of Fayyaa Integrated Development Organization (FIDO)



Tajebesh, 58, a widowed mother of two, near home. She received cash assistance from ZOA with the financial support of EHF to address her needs.

July 2025,
Adiwosene village,
Mai Tsebri town,
Contested Areas.

Photo: UNOCHA/Petro Riabukhin



EHF IN ACTION: RESTORING SAFETY AND DIGNITY FOR WOMEN

She is standing next to her simple home, with a roof held together by four road rocks. She is smiling, but in her eyes, you can see the sadness and the scars of the hardships she endured.

Tajebesh, 58, a widowed mother of two, has lived through years of conflict, insecurity and fear. Her home in Adiwosene village, in Mai Tsebri district, was severely damaged during the 2021 hostilities. For the next four years, she lived alone in a structure with no doors and a leaking roof.

“I was so scared. Every night, as I tried to fall asleep, I would tremble at what the night might bring. Some women around here have suffered from violence, the horrors they have lived through... And in my shelter, without a proper door, anyone could just walk in at any time,” she says.

For many people in Mai Tsebri, humanitarian assistance has been a lifeline. Since last November, Tajebesh and some 2,000 other vulnerable women in the area have received emergency cash assistance. Tajebesh

used the money to buy materials and repair her home, finally securing both her door and roof.

“I feel safe again. Now I can finally sleep, knowing there is a lock on my door,” she shares with tears in her eyes.

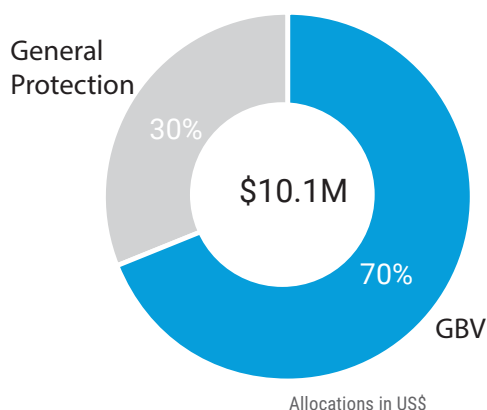
She also used part of the assistance to buy fertilizers for her small maize plot. A modest harvest allows her to meet her family's food needs, and she is able to sell any surplus at a local market.

“I’m saving money to help my daughters. This aid has truly changed my life.”

Programming Highlights

The EHF-supported programming in 2025 demonstrated strong integration across sectors and cross-cutting priorities, including protection mainstreaming, gender equality, inclusion of people with disabilities and community engagement. Multi-sectoral approaches enabled partners to address overlapping needs more effectively.

Gender based violence response



In 2025, the Ethiopia Humanitarian Fund (EHF) continued to support efforts to strengthen protection systems, promote gender equality and respond to gender-based violence (GBV), combining life-saving service delivery with investments in system strengthening and community-based prevention.

The EHF supported 32 projects with a protection component (including GBV), totalling nearly US\$10.1 million, of which nearly 75 per cent was dedicated to GBV prevention, risk mitigation, and response activities. These interventions were implemented across multiple regions of the country, including Amhara, Oromia, Tigray, Somali and Gambela, reflecting the concentration of needs in conflict-affected, displacement-affected and climate-vulnerable areas where protection systems had been significantly weakened and access to services remained limited. Needs assessments across project locations highlighted severe gaps in GBV response, including reported

incidents of conflict-related sexual violence and the widespread lack of access to clinical, psychosocial and legal support.

The EHF-funded interventions prioritized community-based protection approaches, strengthened accountability to affected people and reinforced prevention and response mechanisms related to sexual exploitation and abuse, while also expanding access to comprehensive survivor-centred services such as GBV case management, mental health and psychosocial support (MHPSS), legal assistance, referral services and emergency cash assistance.

MHPSS were delivered at scale, reaching hundreds of thousands of people with combined GBV and psychosocial assistance, including individual counselling, group support sessions and community-based activities, reflecting both the magnitude of needs and the centrality of trauma-informed care in the humanitarian response. Emergency cash assistance was provided to GBV survivors to support access to services, transportation, temporary shelter and other urgent needs, while also helping to reduce negative coping mechanisms linked to economic vulnerability. Across projects, thousands of dignity kits were distributed to women and girls, contributing to improved safety, hygiene and dignity in displacement and crisis settings. These distributions were complemented by the establishment and strengthening of women- and girls-friendly spaces, which provided safe, confidential and accessible entry points for survivors to access integrated services, including psychosocial support, case management and referrals.

In addition to direct service delivery, the EHF investments contributed to strengthening GBV response systems. Projects supported the training of local service providers, including health workers, social workers and legal actors on survivor-centred approaches, Clinical Management of Rape, psychological first aid and

A woman sells essentials
in an improvised shop in
Bakelo camp for internally
displaced people.

May 2025,
Debre Berhan town,
Amhara Region.

Photo: UNOCHA/Charlotte Cans



case management. Referral pathways between health, protection and legal services were established or updated across multiple woredas, improving coordination and enabling more timely and effective support for survivors. Investments in One-Stop Centres and local service structures further enhanced the capacity of national systems to deliver integrated and quality GBV services.

At the community level, interventions placed strong emphasis on prevention and risk mitigation. Large-scale awareness-raising activities were conducted through community dialogues, outreach campaigns and engagement with religious leaders, youth groups and women's associations.

Projects funded by the EHF also supported the establishment of community-based protection committees, strengthened early risk identification, promoted referrals, and facilitated local ownership of GBV prevention efforts. These interventions aimed to reduce stigma, increase awareness of available services and contribute to shifts in harmful social norms that perpetuate GBV. GBV risk mitigation was incorporated across sectors, including shelter, WASH and food security interventions, helping reduce exposure to violence and improve the safety and dignity of affected people.



Women Empowerment-Action (WE-Action), a national women-led NGO, strengthened integrated GBV response services in conflict-affected areas in Amhara by supporting a One-Stop Centre and establishing two women and girls' spaces, providing survivors with access to comprehensive care. The project delivered protection and life-saving assistance to almost 2,000 people, including 530 survivors receiving case management and over 1,000 survivors accessing emergency cash assistance, alongside psychosocial support and "dignity kits" supplies. By combining service delivery with institutional strengthening, the intervention improved access to coordinated medical, psychosocial and protection services, reinforcing survivor-centred approaches in areas with limited service availability and highlighting the critical role of national organizations in delivering specialized protection services.

The EHF-funded projects also adopted inclusive targeting approaches, reaching internally displaced people, returnees and host communities, with particular attention to women and girls, people with disabilities and other vulnerable groups. More people were reached indirectly through improved community

awareness, strengthened service delivery systems and enhanced protection infrastructure, extending the impact of GBV programming beyond direct service recipients. National organizations played a central role in delivering GBV services, particularly in areas with limited humanitarian access.



Union of Ethiopian Women and Children Associations (UEWCA), a national women-led NGO, demonstrated how GBV risk mitigation can be integrated across sectors through multi-sectoral response in conflict-affected woredas of Amhara. The project combined GBV case management, psychosocial support, emergency cash assistance and “dignity kit” distribution with broader humanitarian assistance. In one intervention, almost 12,000 community members were reached through awareness activities, while survivors received tailored support, including referrals and safe access to services. By linking protection with health, WASH and livelihoods support, the intervention helped reduce exposure to risks and strengthen safety and dignity in displacement settings.

Accountability to affected people

Accountability to affected people (AAP) remained a central component of EHF-supported programming in 2025, with partners integrating feedback and complaint mechanisms across sectors and interventions. Community engagement approaches were embedded throughout project design and implementation, ensuring that affected people were informed, consulted and able to influence humanitarian assistance. These efforts were reinforced through partnerships with national and local organizations, including women-led organizations, placing affected communities at the centre of decision-making processes.

The EHF-funded partners applied a range of AAP practices, including community feedback desks, hotline systems, complaint and response mechanisms and regular community consultations. These approaches helped ensure that assistance was aligned with priority needs and adapted to evolving contexts. In several projects, community feedback directly informed programme adjustments, including changes to targeting criteria, delivery modalities, and service locations, thereby improving the relevance and acceptance of assistance, particularly in hard-to-reach and conflict-affected areas. Efforts were made to close the feedback loop by communicating back to communities on actions taken in response to their feedback.



Positive Action for Development (PAD) – a national humanitarian organization – implemented a structured complaints and feedback mechanism in Tigray. The system ensured systematic collection, tracking and response to community feedback through multiple channels, including hotlines, suggestion boxes and community meetings. All cases were registered, categorized and assigned timelines, with urgent and sensitive complaints addressed within 24 hours and standard cases within five days. Feedback was communicated back to communities through preferred channels, ensuring transparency and accountability. The system also enabled analysis of trends and programmatic gaps, directly informing project adjustments and improving the quality and responsiveness of assistance.

Efforts were also made to ensure inclusive access to feedback mechanisms, including outreach to women, older persons and persons with disabilities. Community structures, including leaders and committees in sites for internally displaced people, played an important role in

facilitating communication between partners and affected people, helping ensure that feedback mechanisms were accessible and responsive to diverse needs.



Organization for Social Services, Health and Development (OSSHD) implemented a community-centred accountability and coordination project in Tigray Region, reaching nearly 135,000 people, including internally displaced people, returnees and host communities. The project established structured feedback and complaint mechanisms across 14 displacement sites, including suggestion boxes, complaint committees and community-led coordination platforms, alongside Area-Based Approach communication centres to facilitate access to information and services. The feedback mechanisms were actively used, with some 160 issues resolved. Community governance structures were strengthened through establishing Community Committees, training over 170 committee members, organizing around 40 site-level meetings which enabled internally displaced people to actively participate in decision-making, service monitoring and dispute resolution.

how community-driven systems could be integrated into site management to strengthen accountability and, more broadly, ensure that humanitarian assistance remains aligned with evolving needs.

To strengthen partner capacity and institutionalize AAP practices, the EHF, in collaboration with the AAP Working Group in Ethiopia, organized two rounds of comprehensive AAP capacity-strengthening trainings in Addis Ababa in December 2025. The sessions brought together 48 participants (23 men and 25 women) from 37 national NGOs implementing multi-sectoral EHF-funded projects. The training modules focused on enhancing understanding of AAP principles and minimum standards, and on integrating AAP, prevention of sexual exploitation and abuse, safeguarding, Core Humanitarian Standard commitments, and gender, age and disability inclusion across humanitarian interventions.

These capacity-strengthening efforts resulted in concrete outcomes, with participating organizations developing actionable plans to institutionalize AAP within their systems, strengthen community participation, improve information-sharing practices and establish more inclusive feedback mechanisms. Within this broader effort, partners strengthened their AAP approaches by systematically integrating community engagement into project design and ensuring that information was clear and accessible to support informed participation. During implementation, organizations established safe and inclusive feedback mechanisms, including community committees, hotlines and suggestion boxes, designed in consultation with affected communities to reach vulnerable and marginalized groups.

At the inter-agency level, partners also strengthened coordination through regular

engagement with the AAP Working Group, which brings together almost 70 humanitarian organizations, including participation in monthly meetings and contributions to collective community feedback platforms that recorded more than 2,500 submissions in 2025, of which 93 per cent were addressed through established response pathways. By combining system-wide requirements, targeted capacity building and partner-driven initiatives, the EHF continued to embed accountability as a core component of humanitarian response, ensuring that affected communities play a central role in shaping the assistance they receive.

Rapid response mechanism

In 2025, the EHF-supported interventions contributed to rapid response efforts in areas affected by sudden shocks, including displacement, conflict escalation and disease outbreaks. Through the cost extension of Rapid Response Mechanism (RRM) project started in 2024, the Fund enabled the timely delivery of life-saving assistance in rapidly evolving situations.

The SWAN Consortium – a partnership of Save the Children International, World Vision International, Action Against Hunger, and the Norwegian Refugee Council, together with its national partners, OWS Development Fund (OWS-DF), Fayyaa Integrated Development Organization (FIDO), Ethiopian Red Cross Society (ERCS), Organization for Welfare and Development in Action (OWDA) and Development for Peace Organization (DPO) – continued to be an important RRM in the country. Following the successful implementation of a US\$4 million programme in 2024, the Consortium received a cost extension of \$1.2 million from the EHF in 2025, enabling the activation of 65 response plans, with a total response value of \$1.67 million, to address emergency needs. Critical assistance was delivered across multiple regions, including

A doctor conducts vaccination for livestock to protect animal health and safeguard the livelihoods of pastoralist families who rely on livestock for food and income.

March 2025,
Borena Zone, Oromia Region.

Photo: UNOCHA/Manuel Morini



Amhara, Tigray, Oromia, Afar, Somali, Central Ethiopia, Gambela and Southern Ethiopia. Rapid response interventions focused on delivering integrated multi-sectoral assistance, including emergency shelter, water, sanitation and hygiene, and health and protection services. The ability to mobilize partners quickly and deploy flexible funding was critical in contexts where delays could significantly increase humanitarian risks.

In displacement-affected areas, rapid response support helped address the immediate needs of newly displaced people, including access to basic services and protection. In areas

affected by disease outbreaks, rapid interventions supported containment efforts and continuity of essential services.

The EHF also contributed to the development of a harmonized RRM Operational Framework, led by the RRM coordination function within OCHA Ethiopia. This helped establish streamlined principles, scope, activation triggers and response modalities, strengthening coordination, and improving the effectiveness and predictability of rapid response across partners.



The SWAN Consortium delivered timely assistance to people displaced by an earthquake in Oromia, reaching affected communities within days of the shock. Through its national partner Fayyaa Integrated Development Organization (FIDO), mobile health and nutrition teams provided over 1,200 outpatient consultations, screened nearly 900 children and over 200 pregnant and lactating women for malnutrition, and delivered mental health and psychosocial support to more than 500 people. Additional services included reproductive health care for nearly 400 women and health awareness activities reaching over 2,500 people. The response demonstrates how the EHF-funded partners rapidly deliver integrated life-saving assistance in hard-to-reach areas, combining clinical care, nutrition services and community outreach to address urgent needs in the immediate aftermath of sudden shocks.

Advancing cash and voucher assistance

In 2025, US\$3.6 million of Ethiopia Humanitarian Fund allocations (approximately 8 per cent of total funding) were delivered through cash and voucher assistance (CVA) modalities. Multi-purpose cash accounted for \$1.4 million of CVA programming, followed by conditional cash transfers at \$1.1 million and sector-specific unconditional cash transfers worth over \$950,000. A smaller share of assistance, approximately \$122,000, was delivered through unconditional vouchers.

National NGOs implemented the majority of cash-based programming, managing \$2.86 million of CVA allocations, while international NGOs implemented \$742,000. The strong participation of national partners reflects the Fund's continued commitment to localization and the role of local actors in delivering assistance directly to affected communities, particularly in hard-to-reach and access-constrained areas.

Cash-based assistance supported people in meeting essential needs across multiple sectors, including food security, livelihoods, shelter and other priority humanitarian needs. In displacement-affected and crisis-affected areas, cash assistance enabled families to respond flexibly to rapidly changing needs, while also supporting access to services, including for vulnerable groups such as women and survivors of gender-based violence. By providing resources directly to affected people,

CVA helped preserve dignity and choice while contributing to the local market functionality.

The combination of multi-purpose and conditional cash transfers allowed partners to balance flexibility with targeted support, ensuring that assistance addressed both immediate basic needs and sector-specific priorities. Cash modalities also enabled faster, more adaptable response than in-kind assistance, particularly in contexts where access constraints and logistical challenges limited traditional delivery approaches.

At the same time, the delivery of cash assistance faced operational constraints in some areas, including limited access to formal financial services and banking infrastructure. In such contexts, partners relied on direct cash distribution, which introduced additional operational and protection risks and required strengthened safeguards to ensure safe delivery, including adapted distribution modalities and enhanced risk mitigation measures for people receiving assistance and humanitarian workers.

The EHF continued to coordinate closely with the Cash Working Group and sectors to ensure complementarity with other humanitarian cash interventions and maintain alignment with national CVA standards and transfer values.



Mums for Mums (MfM), an Ethiopian women-led humanitarian organization, implemented an integrated multi-sectoral project supporting internally displaced people returning to Eastern Tigray through deploying mobile teams to hard-to-reach areas. Assistance combined health, nutrition, protection and cash assistance. More than 2,000 families received one-off multi-purpose cash transfers, enabling them to meet immediate needs such as food, shelter and transportation during return and reintegration. Cash assistance was delivered through secure mechanisms, including bank transfers, and complemented by protection services, including risk screening and referral. Overall, the project reached over 50,000 people directly, with at least 250,000 people benefiting indirectly through community-based outreach and awareness activities. Delivered in remote locations with limited infrastructure and logistical challenges, the intervention demonstrated that combining cash assistance with mobile service delivery can improve access to life-saving support and reduce barriers for people in underserved areas, and that cash assistance can be effectively integrated into broader humanitarian response to reduce protection risks and support early recovery.

Women community
members displaced by
conflict.

February 2025,
Denbi Dolo town,
Kelem Wellega,
Oromia Region.

Photo: UNOCHA/Charlotte Cans



EHF IN ACTION: WHEN WOMEN HAVE A SAFE SPACE, HOPE GROWS

For the past three years, Bulbul Town in Gumi Eldello district of Oromia Region has been home to Tena, a mother of six who was once forced to flee her village after conflict broke out in the area. Leaving everything behind was painful, but life in displacement brought new challenges. Violence against women and girls was common, including sexual abuse, yet most cases remained hidden and were endured in silence.

“At that time, we had no place to talk about violence,” Tena says. **“I did not know my rights or where to go if my children or I were harmed. I felt alone, stressed and afraid.”**

Women were rarely included in community discussions or activities. There were no safe spaces for women to meet, learn or support one another. Many, like Tena, remained isolated and without access to information or services.

This began to change through a project supported by the OCHA-managed Ethiopia Humanitarian Fund (EHF), implemented by Mercy Corps together with Action for the Needy Ethiopia (ANE). First, Tena took part in a three-day training on violence against women and girls. She began attending regular activities at the Women and Girls Safe Space in Bulbul Town, facilitated by trained social workers.

“The space became like a second home for us,” she explains. **“It is a place where we feel safe, respected and listened to.”**

In a safe space, women and girls come together to

discuss sensitive issues, access information, and support one another. Tena learned how to prevent violence, how to protect herself and her children, and where to seek help. Through referrals, survivors can access health care, legal assistance, protection services and psychosocial support. Women and girls also participate in skill-building and recreational activities that help rebuild confidence and a sense of normalcy.

Today, Tena is an active participant in the safe space and encourages others to join.

“Now I not only can talk about violence and my rights, but I know exactly what to do if something happens to me or someone I know,” she says. **“This knowledge gives me strength.”**

Through the EHF funding, partners are supporting the establishment of safe spaces and strengthening protection services for women and girls affected by displacement.

These interventions help ensure that survivors have access to information, services and support, while creating spaces where women and girls can reconnect, share experiences and begin to rebuild their sense of safety and dignity. Only in the Oromia region were five similar centres opened in 2025, and 34 more across the country. The EHF allocated US\$10.1 million in 2025 to help nearly 530,000 people with gender-based violence services and support the survivors.

Risk Management

In 2025, the Ethiopia Humanitarian Fund (EHF) continued to apply a comprehensive risk management framework aligned with the CBPF Global Guidelines, ensuring that risks associated with funding, partners and project implementation were systematically identified, monitored and mitigated. Risk management remained fully integrated across the programme cycle, combining capacity assessments, ongoing monitoring and assurance mechanisms, including audits and investigations. This approach enabled the Fund to balance the timely delivery of assistance in high-risk environments with strong accountability and oversight.

Risk management of projects

Throughout the year, the EHF continued to strengthen project assurance through a combination of field monitoring, financial spot checks, oversight of reporting and audits. Monitoring remained risk-based and focused on projects due for review under the operational modality, with priority given to higher-risk grants and contexts.

The EHF conducted almost 90 monitoring visits, of which 80 were required under the Operational Manual, and all 80 were completed, with no monitoring exemptions recorded. By risk category, all required monitoring was completed for 20 high-risk projects, 53 medium-risk projects and 7 low-risk projects. The higher number of actual visits reflected additional follow-up undertaken beyond the minimum required level in some cases.

Financial oversight also remained strong. The EHF conducted 79 financial spot checks, of which 79 were required and completed, with no exemptions. This included all required checks for 16 high-risk projects, 54 medium-risk projects and 9 low-risk projects, indicating a high level of compliance with assurance requirements.

Overall, final reporting compliance remained strong in 2025. A total of 95 narrative reports

(100 per cent target) and 95 financial reports (100 per cent target) were submitted, representing full compliance across all three risk categories.

Audit coverage also remained substantial. In 2025, a total of 88 audits were initiated. Of these, 46 audits were conducted in 2025, while 42 audits were moved to start in early 2026. In 2025, 64 audits were completed, although some had been initiated in previous years. Completed audits included 15 high-risk, 34 medium-risk and 15 low-risk projects. As in previous years, closer coordination between programme, finance and compliance functions supported earlier identification of issues and follow-up on corrective action, including through the triangulation of findings from monitoring, spot checks and audits.

Monitoring activities in 2025 identified a range of operational and compliance-related challenges, consistent with implementation in complex humanitarian contexts. These included (i) delays in project implementation due to access constraints and insecurity, (ii) variations in partner reporting quality and documentation, (iii) procurement and financial management gaps in some projects and (iv) constraints affecting the timely delivery of assistance in remote areas.

All identified issues were systematically followed up through corrective action plans, enhanced monitoring or additional partner support. The Fund used monitoring findings to engage partners as early as possible, agree on corrective actions and strengthen follow-up, including in hard-to-reach and newly accessible areas.

Risk management of partners

The EHF continued to manage partner risk through eligibility procedures, capacity assessments and the ongoing review of partner performance-based scoring. The Fund worked with 47 partners implementing a total allocation envelope of

Asefu, a mother of two who fled violence in Ambo in the Oromia Region, picks up crops near her makeshift home. Supported with a small grant through an NGO funded by the EHF she also managed to launch a small business to sell household essentials in the camp.

May 2025,
Debre Berhan town,
Amhara Region.

Photo: UNOCHA/Charlotte Cans



US\$44.2 million. Partner oversight remained anchored in the Partner Performance Index, with partner risk levels reviewed and updated through the Grant Management System twice during the year. Partners whose risk levels had changed received a detailed account of the reasons therefore. By the end of 2025, the EHF had 22 low-risk partners, 3 national NGOs, 5 international NGOs and 14 UN agencies. Sixty organizations maintained a medium-risk rating: 22 national and 38 international NGOs. Twenty-seven partners were rated as high risk: 24 national and 3 international NGOs.

This performance-based approach allowed the EHF to calibrate oversight measures to partner capacity and track record, while maintaining access to funding for best-placed partners, including national partners operating in complex environments.

As in previous years, partner-related risk management was not limited to compliance control. It also served to identify capacity gaps and tailor support accordingly. Findings from project monitoring, spot checks, audits and partner performance reviews informed targeted follow-up with partners and contributed to

risk-informed decision-making on funding and oversight arrangements.

The number of eligible partners further increased in 2025, marking a stable upward trend towards localization. Following the eligibility process launched in 2024, the EHF conducted 18 capacity assessments of local partners within the 34 previously prioritized applications. By the end of the year, the Fund had 109 eligible partners (49 national NGOs, 46 international NGOs and 14 UN partners)

Risk management of funding

Risk management of funding remained an important element of the Fund's overall assurance framework in 2025. The EHF conducted four training sessions on risk management, oversight, non-compliance and fraud for partners. Twenty-one action plans were developed and monitored to address findings from capacity assessments, monitoring and audits.

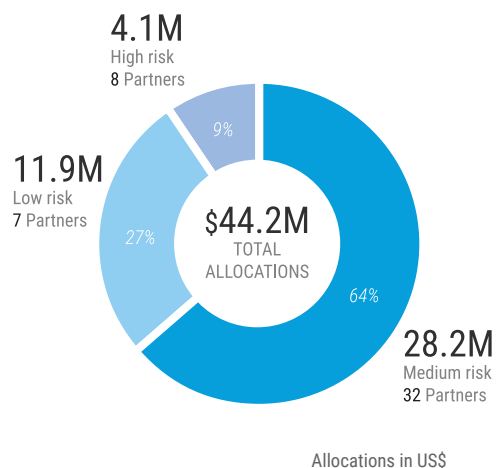
In 2025, seven incidents were reported for Ethiopia across categories such as fraud, destruction of aid, diversion and mismanagement. As in previous years, reported incidents were to be understood as part of the Fund's broader accountability framework, rather than solely as indications of loss or misuse. What remained important was that

incidents were reported, reviewed and addressed in line with CBPF procedures. The Fund maintained a zero-tolerance approach to fraud and misuse of funds, while ensuring that reporting mechanisms remained accessible and trusted by partners.

Building on improvements introduced in previous years, the EHF continued to strengthen its audit planning and oversight processes, including the early initiation of audit firm selection, which contributed to more timely audit completions. Further enhancements in risk management were achieved through closer collaboration across programme, finance and compliance functions, enabling improved triangulation of monitoring data and earlier identification of issues. This approach allowed the Fund to support partners in addressing implementation challenges and to take timely action in cases of non-compliance.

Overall, risk levels in 2025 remained manageable and consistent with the operational context, characterized by access constraints, insecurity and capacity gaps among partners. Key mitigation measures included: risk-based monitoring and prioritization; strengthened partner capacity support; enhanced tracking of recommendations and corrective actions; recovery actions where applicable; strengthened controls to prevent recurrence; and continued investment in training and awareness on compliance and fraud prevention. This approach enabled the EHF to deliver humanitarian assistance at scale while maintaining robust accountability, transparency and oversight mechanisms.

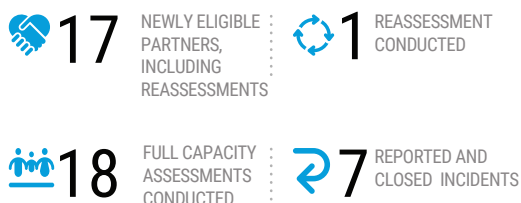
IMPLEMENTATION BY PARTNER RISK LEVEL TYPE



UPDATED RISK LEVEL BASED ON PERFORMANCE INDEX



OVERVIEW 2025



A medical worker measures Rondi's malnutrition levels after completing nutrition treatment as part of community based nutrition interventions by World Vision Ethiopia, funded by EHF.

June 2025,
Shashemene woreda,
West Arsi zone,
Oromia Region.

Photo: World Vision/Bethel Shiferaw



EHF IN ACTION: FIGHTING CHILD MALNUTRITION IN SHALLA DISTRICT. THE STORY OF SAHRA AND RONDİ

When 24-month-old Sahra and Rondi arrived at Fanda Health Centre, they were too weak to eat, their small bodies swollen from severe malnutrition. Sahra's mother, Shaga, feared the worst.

"I thought I would lose my child, but the care given by the health workers and World Vision saved her life," she recalled.

Shalla district in the Oromia Region has a population of nearly 25,000 people. Many children are vulnerable to malnutrition due to recurring drought and limited food availability. Poor feeding practices and frequent childhood illnesses further increase risks, often leading to severe and complicated malnutrition. Stabilization centers play a critical role in reducing deaths among children under five.

With support from the OCHA-managed Ethiopia Humanitarian Fund (EHF), World Vision Ethiopia contributed to the Government's Community-Based Management of Acute Malnutrition (CMAM) programme in the district. Services were provided through eight health centers, each equipped with a stabilization centre for children with severe acute malnutrition and medical complications.

"We provide 24-hour care because these children need close attention," said a nurse at Shalla Health Centre. **"Without stabilization centres, many children would not survive."**

Sahra was admitted to Fanda Health Centre a few months ago in a critical condition. She was weak, with persistent diarrhoea, loss of appetite, skin problems and severe swelling. Shaga, a mother of seven, described the challenges of feeding her family from a small plot of land. Despite her efforts, she was

unable to provide sufficient food to keep Sahra healthy. Sahra remained in the stabilization centre for 12 days, receiving therapeutic milk, routine medical treatment and continuous monitoring. Over time, her condition improved: the swelling reduced, her appetite returned, and she became more active. She was discharged in stable condition and referred to the Outpatient Therapeutic Programme to continue treatment at home. During this time, her mother also received nutrition counselling, hygiene education and caregiver support, including food and small cash assistance.

"When I saw my daughter begin to eat again and smile, I knew she was coming back," Shaga said. **"Today, she is playful and full of energy."**

Rondi, also 24 months old, faced similar challenges. He was referred from Labu Subka village by health extension workers after failing the appetite test and showing signs of severe acute malnutrition, including wasting, diarrhoea and swelling. His mid-upper arm circumference was less than 10 cm, indicating a life-threatening condition. Rondi remained at the stabilization centre for 7 days until his condition improved steadily, and he was discharged to continue treatment through outpatient services, where his progress will be followed.

Through the EHF funding, partners can sustain these life-saving services in areas affected by recurrent drought and food insecurity, ensuring that children with severe acute malnutrition receive timely treatment. By strengthening facility-based care alongside community-level detection and referral, the intervention contributes to reducing preventable deaths and improving recovery outcomes for the most vulnerable children.

Annexes

Annexes list		Annexes title
32	ANNEX A	2025 EHF ADVISORY BOARD
33	ANNEX B	COMMON PERFORMANCE FRAMEWORK

ANNEX A

2025 EHF ADVISORY BOARD

STAKEHOLDER	ORGANIZATION
Chairperson	UN Resident and Humanitarian Coordinator (RC/HC)
INGO	Humanitarian INGO Forum (HINGO)
INGO	International Orthodox Christian Charities (IOCC)
INGO	ActionAid
INGO	World Vision
NNGO	Fayyaa Integrated Development Organization (FIDO)
NNGO	Union of Ethiopian Women and Children Associations (UEWCA)
NNGO	Women Empowerment Action (WE-Action)
UN	Food and Agriculture Organization (FAO)
UN	International Organization for Migration (IOM)
UN	World Food Programme (WFP)
Donor	Italy
Donor	Luxemburg
Donor	Ireland
Observer	U.S. Agency for International Development (USAID)
EHF/OCHA	United Nations Office for the Coordination of Humanitarian Affairs (OCHA)

ANNEX B

COMMON PERFORMANCE FRAMEWORK

The CBPFs measures its performance against a management tool that provides a set of indicators to assess how well a Fund performs in relation to the policy objectives and operational standards set out in the CBPF Global Guidelines. This common methodology enables management and stakeholders involved in the governance of the Funds to identify, analyze and address challenges in reaching and maintaining a well-performing CBPF.

CBPFs embody the fundamental humanitarian principles of humanity, impartiality, neutrality and independence, and function according to a set of specific principles: Inclusiveness, Flexibility, Timeliness, Efficiency, Accountability and Risk Management.





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